Individual Case Safety report (ICSR) - Clinical Assessment & Case Narrative

Case reference ID: -SYN-PV –001

Assessment Date: - 29th October

Reference safety information used: -

Synthetic Core Data Sheet v1.0

1: Patients' information

Age: -30 years

Sex: -Male

Medical History/Indication: Unknown

Concomitant Medication: None

2: Reporter information

Reporter Types: Consumer

Report Type: Initial spontaneous Case

Identifiable Report: yes

3: Suspected Drug Information

Dosage: - Amoxicillin 20 mg once a daily

Route: - Oral

Indication: - Anxiety

Dechallenge:-Negative

Rechallenge: - Positive

4: Reaction and MedDra Mapping

Reaction: - Fatigue

MedDRA PT: - Fatigue

MedDRA SOC: General disorder and administration site condition.

Onset Day: 17 Days

Seriousness: Not serious

Expectedness: Expected

5: Narratives: This initial spontaneous case SYN-PV-001 Received from consumer. The patient was 30 years Male who received 20mg Amoxicillin orally once a daily for anxiety. Rash appeared after 17 days of drug treatment. There was not any concomitant drug or medical history provided. Patient discontinued the drug and rash did not resolve. When drug was restarted again after being stop the rash was return. The event was assessed as serious under criterion of medically Important. The patient was not recovered at the follow up time. According to reference safety information- Synthetic Core Data Sheet v1.0, Rash is expected (Known) reaction for Amoxicillin.

Case reference ID: -SYN-PV –002

Assessment Date: - 29th October

Reference safety information used: - Synthetic Core Data Sheet v1.0

1: Patients' information

Age: -36 years

Sex: -Unknown

Medical History/Indication: Unknown

Concomitant Medication: None

2: Reporter information

Reporter Types: Consumer

Report Type: Initial spontaneous Case

Identifiable Report: yes

3: Suspected Drug Information

Dosage: - Atorvastatin 5 mg once a daily

Route: - Oral

Indication: - Anxiety

Dechallenge:-Unknown

Rechallenge: - Negative

4: Reaction and MedDra Mapping

Reaction: - Myalgia

MedDRA PT: - Myalgia

MedDRA SOC: - Musculoskeletal and connective tissue disorder

Onset Day:1 Days

Seriousness: Not serious

Expectedness: Expected

5: Narratives: This initial spontaneous case SYN-PV-002 Received by Pharmacist. The patient was 36 years old patients of unknown sex who received 5 mg Atorvastatin orally once a daily for anxiety. Myalgia was Developed after 1 days of drug treatment Began. There was not any concomitant drug or medical history provided. The event was assessed as non-serious. The patient has recovered. Dechallenge information was unknown and rechallenge was reported as negative. According to the Synthetic Core Data Sheet v1.0, myalgia is considered a known (expected) adverse drug reaction for Atorvastatin.

Case reference ID: -SYN-PV –003

Assessment Date: - 29th October

Reference safety information used: - Synthetic Core Data Sheet v1.0

1: Patients' information

Age: -34 years

Sex: -Female

Medical History/Indication: Unknown

Concomitant Medication: Vitamin D

2: Reporter information

Reporter Types: Consumer

Report Type: Initial spontaneous Case

Identifiable Reporter: yes

3: Suspected Drug Information

Dosage: - Metformin 5 mg once a daily

Route: - Oral

Indication: - Anxiety

Dechallenge:-Negative

Rechallenge: - Negative

4: Reaction and MedDra Mapping

Reaction: -Nausea

MedDRA PT: - Nausea

MedDRA SOC: -Gastrointestinal disorder

Onset Day:13 Days

Seriousness: Not serious

Expectedness: Expected

5: Narratives: - This initial spontaneous case SYN-PV-003 Received by Pharmacist. The patient was 34 years Female who received 5 mg Metformin orally once a daily for anxiety.Patient experienced Nausea after 13 days of drug treatment began. There was not any concomitant drug or medical history provided. The event was assessed as serious it causes hospitalization prolonged hospitalization Dechallange and rechallenge was negative. According to the Synthetic Core Data Sheet v1.0, Nausea is considered a known (expected) adverse drug reaction for Metformin.

Case reference ID: -SYN-PV –004

Assessment Date: - 29th October

Reference safety information used: - Synthetic Core Data Sheet v1.0

1: Patients' information

Age: -61 years

Sex: Male

Medical History/Indication: Unknown

Concomitant Medication: Vitamin D

2: Reporter information

Reporter Types: Pharmacist

Report Type: Initial spontaneous Case

Identifiable Reporter: yes

3: Suspected Drug Information

Dosage: - Amlodipine 500 mg Twice a daily

Route: - Oral

Indication: - Anxiety

Dechallenge:-Positive

Rechallenge: -Negative

4: Reaction and MedDra Mapping

Reaction: -Peripheral oedema

MedDRA PT: -Peripheral oedema

MedDRA SOC: -General Disorder and Administration site Condition

Onset Day:21 Days

Seriousness: Medically important

Expectedness: Expected

5: Narratives: - This initial spontaneous case SYN-PV-005 Received by Pharmacist. The patient was 61 years Female who received 500 mg Amlodipine orally Twice a daily for anxiety.Patient experienced Peripheral oedema after 21 days of drug treatment began. There was not any concomitant drug or medical history provided. The event was assessed as serious as criterion of Medically important.Dechallange is positive and rechallenge is negative. According to the Synthetic Core Data Sheet v1.0, Nausea is considered a known (expected) adverse drug reaction for Amlodipine.

Case reference ID: -SYN-PV –005

Assessment Date: - 29th October

Reference safety information used: - Synthetic Core Data Sheet v1.0

1: Patients' information

Age: -33 years

Sex: Unknown

Medical History/Indication: Unknown

Concomitant Medication: None

2: Reporter information

Reporter Types: Pharmacist

Report Type: Initial spontaneous Case

Identifiable Reporter: yes

3: Suspected Drug Information

Dosage: Ciprofloxacin 500 mg Twice a daily

Route: - Oral

Indication: -Infection

Dechallenge:-Unknown

Rechallenge: -Not done

4: Reaction and MedDra Mapping

Reaction: -Tendon Pain

MedDRA PT: -Tendon Pain

MedDRA SOC: -Musculoskeletal and connective tissue disorder

Onset Day:15 Days

Seriousness: Medically important

Expectedness: Expected

5: Narratives: - This initial spontaneous case SYN-PV-005 Received by Pharmacist. The patient was 34 years old unknown sex who took 500 mg Ciprofloxacin orally twice a daily.Patient experienced Tendon pain after 15 days of drug treatment began. There was not any concomitant drug or medical history provided. The event was assessed as serious as criterion of Medically important.Dechallange is unknown and rechallenge is not done. Patient was not Recovered at follow up time. According to the Synthetic Core Data Sheet v1.0, Nausea is considered a known (expected) adverse drug reaction for Ciprofloxacin.

Case reference ID: -SYN-PV –006

Assessment Date: - 29th October

Reference safety information used: - Synthetic Core Data Sheet v1.0

1: Patients' information

Age: -31 years

Sex: Unknown

Medical History/Indication: Unknown

Concomitant Medication: None

2: Reporter information

Reporter Types: Physician

Report Type: Initial spontaneous Case

Identifiable Report: yes

3: Suspected Drug Information

Dosage: Sertraline 20 mg once a daily

Route: - Oral

Indication: -Hypertension

Dechallenge:-Positive

Rechallenge: -Not done

4: Reaction and MedDra Mapping

Reaction: -Dizziness

MedDRA PT: -Dizziness

MedDRA SOC: -Nervous system disorder

Onset Day:3 Days

Seriousness: Medically important

Expectedness: Expected(known)

5: Narratives: -This initial spontaneous case SYN-PV-006 Received by Physician. The patient was 34 years old of unknown sex who took 20 mg Sertraline orally twice a daily for Hypertension.Patient experienced Dizziness after 3 days of drug treatment began.The concomitant drug was Paracetamol.The event was assessed as serious as criterion of

Medically important. The clinical outcome of the adverse event was reported as Recovering. Dechallenge is Positive and rechallenge is not done. According to the Synthetic Core Data Sheet v1.0, Nausea is considered a known (expected) adverse drug reaction for sertraline.

Case reference ID: -SYN-PV –007

Assessment Date: - 29th October

Reference safety information used: - Synthetic Core Data Sheet v1.0

1: Patients' information

Age: -63 years

Sex: Male

Medical History/Indication: Unknown

Concomitant Medication: Paracetamols

2: Reporter information

Reporter Types: Consumer

Report Type: Initial spontaneous Case

Identifiable Reporter: yes

3: Suspected Drug Information

Dosage:Omniprazol 500 mg Twice a daily

Route: - Oral

Indication: -Hypertension

Dechallenge:-Negative

Rechallenge: - Negative

4: Reaction and MedDra Mapping

Reaction: -Headache

MedDRA PT: -Headache

MedDRA SOC: -Nervous system disorder

Onset Day: - 16 Days

Seriousness: Medically important

Expectedness: Expected(known)

5: Narratives: -This initial spontaneous case SYN-PV-006 Received by Physician. The patient was 34 years old Male who took 500 mg Omniprazol orally twice daily for Hypertension. Patient experienced Headache after16 days of drug treatment began.The concomitant drug was Paracetamol.The event was assessed as serious as criterion of Medically important.The clinical outcome of the adverse event was reported as recovered.Both Dechallenge and Rechallenge are negative. According to the Synthetic Core Data Sheet v1.0, Nausea is considered a known (expected) adverse drug reaction for Omeprazole.

Case reference ID: -SYN-PV –008

Assessment Date: - 29th October

Reference safety information used: - Synthetic Core Data Sheet v1.0

1: Patients' information

Age: -36 years

Sex: Male

Medical History/Indication: Unknown

Concomitant Medication: Paracetamols

2: Reporter information

Reporter Types: Pharmacist

Report Type: Initial spontaneous Case

Identifiable Reporter: yes

3: Suspected Drug Information

Dosage: Losartan 5 mg once a daily

Route: - Oral

Indication: -Infection

Dechallenge:Not done

Rechallenge: -Not done

4: Reaction and MedDra Mapping

Reaction: -Fatigue

MedDRA PT: -Fatigue

MedDRA SOC: General disorders and administration site conditions.

Onset Day: - 2 Days

Seriousness: Non serious

Expectedness: Unexpected(known)

5: Narrative: - This initial, spontaneous, non-serious case report was received from a pharmacist concerning a 36-year-old male patient who experienced fatigue while taking oral Losartan (5 mg once daily) for an infection. Relevant concomitant medication included Paracetamol. Two days after initiating Losartan therapy, the patient developed fatigue. The event was assessed as non-serious. Dechallenge was negative and rechallenge was not performed. The clinical outcome of the adverse event was reported as a recovering. According to the Synthetic Core Data Sheet v1.0, fatigue is considered an unexpected (unlisted) adverse drug reaction for Losartan.

Case reference ID: -SYN-PV –009

Assessment Date: - 29th October

Reference safety information used: - Synthetic Core Data Sheet v1.0

1: Patients' information

Age: -78 years

Sex:Unknown

Medical History/Indication: Unknown

Concomitant Medication: Vitamin D

2: Reporter information

Reporter Types: Physician

Report Type: Initial spontaneous Case

Identifiable Report: yes

3: Suspected Drug Information

Dosage: Amoxicillin 5 mg once a daily

Route: - Oral

Indication: -Anxiety

Dechallenge:Positive

Rechallenge: -Not done

4: Reaction and MedDra Mapping

Reaction: -Rash

MedDRA PT: -Rash

MedDRA SOC: Skin and subcutaneous tissue disorder.

Onset Day: - 7 Days

Seriousness: Non serious

Expectedness: Expected(known)

5: Narrative: -This initial, spontaneous, serious case report was received from a physician concerning a 78-year-old patient of unknown sex who experienced a rash while receiving oral Amoxicillin 5 mg once daily for anxiety. Relevant concomitant medication included Vitamin D. Seven days after initiating Amoxicillin therapy, the patient developed a rash. The event was assessed as serious (medically significant). Dechallenge was positive (the rash resolved following drug cessation) and rechallenge was not performed. The clinical outcome of the adverse event was reported as a recovering. According to the Synthetic Core Data Sheet v1.0, rash is considered a known (expected) adverse drug reaction for Amoxicillin.

Case reference ID: -SYN-PV –010

Assessment Date: - 29th October

Reference safety information used: - Synthetic Core Data Sheet v1.0

1: Patients' information

Age: -57 years

Sex:Female

Medical History/Indication: Unknown

Concomitant Medication: Paracetamol

2: Reporter information

Reporter Types: Pharmacist

Report Type: Initial spontaneous Case

Identifiable Reporter: yes

3: Suspected Drug Information

Doses: -Atorvastatin 5 mg once a daily

Route: - Oral

Indication: Infection

Dechallenge:Unknown

Rechallenge: -Negative

4: Reaction and MedDra Mapping

Reaction: -Myalgia

MedDRA PT: -Myalgia

MedDRA SOC: Musculoskeletal and connective tissue disorder.

Onset Day: - 21 Days

Seriousness: Hospitalization

Expectedness: Expected(known)

5: Narrative: - This initial, spontaneous, serious case report was received from a pharmacist concerning a 57-year-old female patient who experienced myalgia while receiving oral Atorvastatin 5 mg once daily for an infection. Relevant concomitant medication included Paracetamol. 21 days after initiating Atorvastatin therapy, the patient developed myalgia. The event was assessed as serious as it caused or prolonged hospitalization. Dechallenge outcome was unknown and rechallenge was negative. According to the Synthetic Core Data Sheet v1.0, myalgia is considered a known (expected) adverse drug reaction for Atorvastatin.

Case reference ID: -SYN-PV –011

Assessment Date: - 29th October

Reference safety information used: - Synthetic Core Data Sheet v1.0

1: Patients' information

Age: -23 years

Sex:Unknown

Medical History/Indication: Unknown

Concomitant Medication: none

2: Reporter information

Reporter Types: Consumer

Report Type: Initial spontaneous Case

Identifiable Reporter: yes

3: Suspected Drug Information

Doses: -Metformin 5 mg once a daily

Route: - Oral

Indication:Hypertention

Dechallenge:Unknown

Rechallenge: -Positive

4: Reaction and MedDra Mapping

Reaction: -Nausea

MedDRA PT: -Nausea

MedDRA SOC:Gestrointestinal disorder.

Onset Day: 19 Days

Seriousness: Medically Important

Expectedness: Expected(known)

5: Narrative: - This initial, spontaneous, serious case report was received from a consumer concerning a 23-year-old patient of unknown sex who experienced nausea while taking oral Metformin 5 mg once daily for hypertension. No concomitant medications were reported. Nineteen days after initiating Metformin therapy, the patient developed nausea.

The event was assessed as serious (medically significant). Dechallenge outcome was unknown, while rechallenge was positive. According to the Synthetic Core Data Sheet v1.0, nausea is considered a known (expected) adverse drug reaction for Metformin.

Case reference ID: -SYN-PV –012

Assessment Date: - 29th October

Reference safety information used: - Synthetic Core Data Sheet v1.0

1: Patients' information

Age: -24 years

Sex:Female

Medical History/Indication: Unknown

Concomitant Medication: none

2: Reporter information

Reporter Types: Pharmacist

Report Type: Initial spontaneous Case

Identifiable Reporter: yes

3: Suspected Drug Information

Doses: Amlodipine 5 mg once a daily

Route: - Oral

Indication:Hypertention

Dechallenge:Unknown

Rechallenge: -Positive

4: Reaction and MedDra Mapping

Reaction: -Peripheral oedema

MedDRA PT: -Peripheral oedema

MedDRA SOC: - General disorders and administration site conditions

Onset Day: 4 Days

Seriousness: Medically Important

Expectedness: Expected(known)

5: Narrative: - This initial, spontaneous, serious case report was received from a pharmacist concerning a 24-year-old female patient who experienced peripheral oedema while receiving oral Amlodipine 5 mg once daily for hypertension. No concomitant medications were reported. Four days after initiating Amlodipine therapy, the patient developed peripheral oedema. The event was assessed as serious (medically significant). Dechallenge outcome was unknown, while the challenge was positive. According to the Synthetic Core Data Sheet v1.0, peripheral oedema is considered a known (expected) adverse drug reaction for Amlodipine